

HCA Healthcare Transformation Roadmap

Eduardo Donaire Filho | Member A | Group 2 | Week 6

We recommend a gated rollout: prepare the data, local systems and AI controls; test three staff-facing tools in limited settings; expand only after measured results and clinical approval. This document supplies the shared initiative names, visual roadmap, dependencies and phase rationales for the team’s RACI and QBR dashboard.

Case and evidence boundary

The group selected HCA Healthcare over Domino’s and BayCare. HCA keeps the healthcare context and offers a substantive 2023 Investor Day presentation. Domino’s was the alternative for clearer outcome reporting; BayCare offered continuity but a thinner investor-document base. Our evidence window is 2021–2024. This is an analytical reconstruction, not HCA’s official roadmap or a claim about its October 2026 deployment status.

Six published initiatives

ID and shared name	What the source supports
I1 Google Cloud data platform	2021 partnership for secure data and analytics; data integration also appears in the Investor Day deck. [S1, S2]
I2 MEDITECH Expanse	Investor Day describes enterprise conversion over approximately five years, with standardized workflows and data. [S1, slide 107]
I3 Responsible AI Program	The 2024 Impact Report describes an established program for AI standards, oversight and responsible use. [S4, pp. 14–15]
I4 Timpani nurse scheduling	Named labor-management tool in Investor Day; the Impact Report documents limited deployment and planned expansion. [S1, slide 108; S4, p. 16]
I5 Augmedix clinical documentation	Investor Day names the proof of concept. HCA reported an early pilot with about 75 ER physicians at four hospitals. [S1, slide 109; S3]
I6 Nurse handoff assistant	Shift handoff appears in Investor Day. HCA reported testing a generative-AI handoff tool at UCF Lake Nona Hospital. [S1, slides 110–112; S3]

Counting rule: six distinct initiatives. I4–I6 appear in both Walk and Run because scale follows a pilot; these are not counted as three additional initiatives. Quarter ranges, scopes, gates and targets below are group proposals, not reported HCA commitments.

Visual roadmap and dependencies

Relative plan: Q1–Q3 = months 1–9; Q4–Q6 = months 10–18; Q7–Q10 = months 19–30. Each initiative advances on evidence, not simply because a quarter ends.

Initiative	Crawl Q1–Q3	Walk Q4–Q6	Run Q7–Q10
I1 Google Cloud data platform	Secure pilot data feeds, access controls and quality checks.	Operate and monitor pilot feeds.	Extend validated feeds by site wave.
I2 MEDITECH Expanse	Local workflow and interface readiness for pilot sites.	Continue broader EHR migration in parallel.	Continue migration; do not claim completion in 30 months.
I3 Responsible AI Program	Owners, review rules, change control and incident response.	Approve pilots and monitor changes.	Reapprove each new site and model version.
I4 Timpani nurse scheduling	—	I4-W: limited scheduling deployment.	I4-R: enterprise scheduling rollout by eligible site waves.
I5 Augmedix clinical documentation	—	I5-W: limited ER documentation deployment.	I5-R: scale across eligible HCA emergency departments.
I6 Nurse handoff assistant	—	I6-W: one-hospital handoff deployment.	I6-R: scale across eligible inpatient nursing units.

Named capability dependencies

I1 Google Cloud data platform → I5 Augmedix clinical documentation and I6 Nurse handoff assistant: supplies approved, traceable clinical data and secure access. This is our proposed common architecture, not a claim that every vendor uses the same backend.

I2 MEDITECH Expanse local readiness → I4 Timpani nurse scheduling, I5 Augmedix clinical documentation and I6 Nurse handoff assistant: supplies validated local workflow mappings and interfaces. Timpani also requires workforce and timekeeping feeds. Existing EHR interfaces may qualify; completing enterprise Expanse conversion is not a prerequisite.

I3 Responsible AI Program → I4 Timpani nurse scheduling, I5 Augmedix clinical documentation and I6 Nurse handoff assistant: supplies named oversight, human review and controlled model changes.

I4-W → I4-R: proves usable schedules and less preparation work. I5-W → I5-R: proves documentation quality and time benefit. I6-W → I6-R: proves handoff completeness and adoption. Each transition requires the gates on the next page.

Crawl parallelism: I1, I2 and I3 can start together. None requires another Crawl initiative to finish. The nine-month scope is readiness for selected sites, not full EHR replacement.

Pilot gates and scale requirements

All thresholds on this page are proposed classroom planning assumptions. Establish a four-week local baseline before each pilot and compare comparable units, workloads and shifts. Clinical leaders must approve the safety criteria before use.

I4 Timpani nurse scheduling

Walk scope: two nursing units in each of three hospitals for a 12-week evaluation. Dependencies: I2 local workflow readiness and I3 oversight, plus validated workforce feeds. Readiness outcome: at least 20% less schedule-preparation time than baseline, without worse staffing-rule compliance. At least 80% of eligible nurse managers must actively use the tool.

Run scope: expand I4-W across eligible nursing units in all participating HCA hospital divisions, in waves. Required organizational change: a common scheduling policy, trained local nurse-manager champions, a workforce-data owner and a formal path for local exceptions. A new site enters only after its own readiness checks.

I5 Augmedix clinical documentation

Walk scope: approximately 75 volunteer ER physicians across four hospitals, using the historical pilot's scale as a planning reference. Dependencies: I1 secure data access, I2 local EHR integration and I3 clinical oversight. Readiness outcome: at least 20% less documentation time per encounter over 12 weeks, with no deterioration in audited note quality and physician approval of every final note.

Run scope: expand I5-W to eligible emergency departments across HCA. Required organizational change: department-level training, a physician owner for note quality, protected training time and ongoing chart audits. Expansion beyond emergency care needs a separate evaluation and is not assumed here.

I6 Nurse handoff assistant

Walk scope: two inpatient units at one hospital for 12 weeks, using the published single-hospital test as a scope reference. Dependencies: I1 clinical data, I2 local workflow integration and I3 review controls. Readiness outcome: at least 15% less handoff-preparation time, no deterioration in audited completeness, and nurse review of every generated handoff.

Run scope: expand I6-W across eligible inpatient nursing units in participating HCA divisions. Required organizational change: a standard handoff checklist, nurse educators, shift-level training and escalation rules. The tool supports, rather than replaces, nurse-to-nurse communication.

Pause and release decisions

An unresolved critical safety or privacy incident pauses the affected pilot or rollout wave. Missed benefit or adoption targets trigger a review before additional funding. Passing a time-saving target never overrides a clinical safety finding. These are proposed governance rules, not claims about HCA's existing policy.

Phase rationales

Crawl rationale

Crawl creates the conditions for a reliable test. Secure data access, local EHR integration and responsible AI controls can begin in parallel because each has a separate deliverable. We limit this phase to readiness at selected hospitals, not completion of the enterprise Expanse migration. That distinction keeps the foundation achievable within nine months. Without these capabilities, a pilot could confuse missing data, inconsistent workflows or unclear ownership with a failure of the tool itself. Walk begins only when the relevant site has passed its readiness checks.

Walk rationale

Walk tests whether the tools help real staff in a controlled setting. Timpani, clinical documentation and nurse handoff use the foundation differently, so each needs its own baseline, limited deployment and measured result. A favorable demonstration is not enough. The pilots must show less administrative work without weaker clinical quality or staffing compliance, and users must actually adopt the tools. These checks justify later investment and reveal training or workflow changes while the exposure is still small. Only a successful pilot moves into its corresponding Run rollout.

Run rationale

Run expands proven deployments rather than introducing untested use cases. Each rollout inherits its pilot's evidence, but a new hospital still needs local data, training and safety checks. Enterprise standards create consistency, while an exception process recognizes that hospitals and units differ. Scaling therefore requires organizational changes: local champions, clear ownership, continuing audits and support for staff. We release investment in waves and pause when quality, adoption or benefit targets are missed. This prevents a large installation count from being mistaken for a successful transformation.

What this roadmap does not claim

The proposed phases are a dependency-based interpretation of published initiatives, not HCA's historical calendar. Published pilot or rollout counts demonstrate activity, not proven financial or clinical outcomes. We do not use the unverified September 2026 hospital count, assume cost savings, or describe Run as already complete.

Why the sequence matters

The central risk is scaling a useful-looking tool before local data, staff adoption and clinical review are ready. The practical response is to fund readiness first, collect evidence in a limited setting, and give leaders explicit stop or expand decisions at every gate.

Handoff for the RACI and QBR

Eduardo owns the roadmap and final shared initiative inventory. Cole and Alifiya own the RACI. Gerald owns the QBR dashboard. The following suggestions connect those deliverables; they do not replace Members B and C’s analysis.

Recurring decisions for Cole and Alifiya

Decision to assign in the RACI	Roadmap connection
Approve quarterly funding and site-wave priorities	I1–I6; foundation readiness versus rapid rollout
Approve architecture or local workflow exceptions	I1 and I2; common standards versus local needs
Resolve conflicts between division and enterprise priorities	I4-R, I5-R and I6-R rollout sequencing
Assign data ownership and approve access or quality remediation	I1 and I2; clinical and workforce data
Approve model changes, pause decisions and release gates	I3 and every Walk-to-Run transition
Approve adoption remediation and training resources	I4–I6; underused tools or failed benefit gates

Use realistic, source-checked HCA role titles and exactly one Accountable per decision. These are proposed decision rights, not a description of HCA’s actual internal RACI.

Candidate metrics for Gerald

Metric and phase	Type	Proposed target
I1 pilot feeds passing quality checks Crawl	Leading	100% of required feeds before pilot release
I2 pilot sites passing integration checks Crawl	Leading	100% of selected pilot sites
I3 approved oversight and review plans Crawl	Leading	100% of pilot use cases
I4–I6 active trained users / eligible users Walk	Leading and adoption	At least 80%, reported separately per tool
I4 schedule-preparation minutes Walk	Lagging	At least 20% below matched baseline
I5 documentation minutes per encounter Walk	Lagging	At least 20% below matched baseline
I6 handoff-preparation minutes Walk	Lagging	At least 15% below matched baseline
I4–I6 rollout waves passing all release gates Run	Leading	100% before the next wave

Actuals are not public: show “Not measured” until collected, not invented green status. Display safety and quality as release constraints alongside speed metrics. B and C should link 1–3 quarterly decisions to the RACI, for example funding the next wave or pausing a failed pilot.

Sources and final review

Primary documents were inspected for the claims below. Public evidence supports initiative existence and selected historical scope; proposed targets and dependencies are our analytical work. The group must personally verify the underlying sources before submission, as required by the assignment.

S1 HCA Healthcare Investor Day 2023

November 9, 2023. Use printed slides 107–112: Expanse, Timpani, Augmedix proof of concept and shift handoff. Slide 107 describes an approximately five-year enterprise Expanse conversion. The deck is the main investor-document anchor, not an AI-generated case summary.

[Open the full Investor Day presentation](#)

S2 HCA Healthcare Google Cloud partnership

May 26, 2021. HCA's investor-site announcement documents the secure data and analytics partnership. It establishes the foundation initiative and its date, not completed transformation outcomes.

[Open the 2021 partnership announcement](#)

S3 HCA Healthcare generative AI collaboration

August 29, 2023. HCA's investor-site announcement supplies the historical documentation pilot scope and the nurse handoff test at UCF Lake Nona. Human review remains part of the documented workflow.

[Open the 2023 clinical AI announcement](#)

S4 HCA Healthcare Annual Impact Report 2024

Printed pages 14–16. Responsible AI is described on pages 14–15. Page 16 reports Timpani in nine facilities and another 21 hospitals planned for early 2024. Those figures are dated context, not current deployment counts or evidence of realized savings.

[Open the 2024 Annual Impact Report](#)

Before the group submits

- Confirm the six initiative IDs and names across the roadmap, RACI and dashboard.
- Approve or revise the proposed timeline, pilot scopes, targets and pause rules as a group.
- Check that every Run expansion names its Walk pilot and the organizational change needed.
- Use measured or explicitly illustrative dashboard data. Do not present assumptions as HCA results.
- Personally check the cited pages and rehearse the handoff from A to B to C.

Prepared with AI assistance for drafting and formatting. Source verification and final analytical decisions remain the group's responsibility.